

Risk Summary

Preeclampsia

Medium Score: 0.6395082154771051

Model Interpretation: Risk-based triage only (not diagnosis).

Macrosomia

Medium Score: 0.5943816940585895

Model Interpretation: Risk-based coaching support.

Summary: The patient is at 32 weeks gestational age with a medium risk for preeclampsia and a medium risk for macrosomia. Key concerns include an elevated systolic blood pressure of 142 mmHg and the presence of proteinuria (1+ dipstick), indicating potential hypertensive issues in pregnancy. Additionally, the patient has a high pre-pregnancy BMI and a fasting glucose of 99 mg/dL, which, while GDM is not diagnosed, warrants close monitoring.

Patient Snapshot (Recent Inputs)

Parameter	Value	Notes
Blood Pressure	142/79 mmHg	Hypertensive threshold ≥140/90, severe ≥160/110.
Proteinuria (dipstick)	1	≥1+ supports PE suspicion in context of HTN.
Symptoms	Headache=0, Vision=0, RUQ Pain=0, Swelling=0	Severe features increase escalation priority.
Glucose	Fasting=99 mg/dL 1hr PP=126 mg/dL	Targets typically fasting <95, 1hr <140 (pregnancy goals).
HbA1c	5.6	Higher HbA1c indicates sustained glycemic elevation.
Maternal Context	Age=38 BMI=42.4 GDM=0	Risk modifiers for fetal overgrowth + metabolic risk.
Activity / Sleep	Post-meal walk=29 min Sleep=6.1 hrs	Lifestyle factors influence glycemic response.
Food Log	paneer salad	Used for dietary correlation patterns.

Reason for Alert

Action Requested: ALERT_DOCTOR

Clinical Rationale: ALERT: Patient ID TEST_MED_MED (32 weeks gestational age) has a medium preeclampsia risk tier and medium macrosomia risk tier. Key vitals include systolic BP 142 mmHg, diastolic BP 79 mmHg, and proteinuria dipstick 1. Fasting glucose is 99 mg/dL, and pre-pregnancy BMI was 42.4. Patient reports no severe headache, vision changes, upper abdominal pain, or swelling of face/hands. Given the elevated systolic BP and proteinuria, further clinical evaluation for preeclampsia is recommended.

Suggested Next Steps

- Review trend: BP, symptoms, proteinuria, glucose readings
- Consider confirmatory tests (protein/creatinine ratio, LFTs, platelets) if clinically indicated
- Consider GDM diet review or insulin adjustment if glucose targets exceeded
- Provide patient education + return precautions

